

PROGRESS NOTE

Date: 16/04/2026

Time: 9:00 AM

Doctor: Dr. [unclear]

DIAGNOSIS (UPDATED):

Hypertensive Urgency

(Note: Previous note stated Hypertensive Emergency - please clarify with treating team)

SUBJECTIVE:

Patient reports headache has reduced significantly.

Vision improving. No vomiting since last night.

Tolerating oral feeds well.

OBJECTIVE:

PR: 84 bpm

BP: 170/100 mmHg (improved from 210/120 mmHg)

RR: 16/min

SP02: 98% on room air

Temperature: 98.4 F

CNS: Conscious and oriented. No focal deficit.

CVS: S1 S2 heard. No murmurs.

PA: Soft, non-tender.

INVESTIGATIONS REVIEWED:

CBC: Haemoglobin 13.2 gm/dL, TLC 8200 cells/cumm,

Platelets 2.1 lakhs/cumm - NORMAL

Serum Creatinine: 1.8 mg/dL (elevated - reference 0.7-1.4)

Serum Sodium: 138 mmol/L (normal)

Serum Potassium: 3.6 mmol/L (normal)

ECG: Left ventricular hypertrophy pattern noted

CT Brain: No intracranial bleed. No infarct.

Mild periventricular white matter changes noted.

Urine Routine: Protein ++, RBC 2-3/hpf

RFT Complete Report: PENDING - sample sent 15/04/2026 7pm

PLAN:

- Tab. Amlodipine increased to 10mg OD
(dose change - no reason documented)
- Tab. Telmisartan 40mg OD added
- Tab. Furosemide 40mg OD added
- Continue Tab. Metformin 500mg BD
- Nephrology opinion sought - pyelonephritis vs hypertensive nephropathy (conflicting impression)
- Repeat RFT after 48 hours
- Continue BP monitoring

Doctor Signature: [unclear]